

INSTITUTIONAL SILENCE

A Documentary in Real Time

Based on the True Story of

WILBUR WILLIAM MATTHÉE

Specialist Nurse • Whistleblower • NQF Level 8 • Self-Represented Litigant

VS.

AFROCENTRIC HEALTH / MEDSCHEME HOLDINGS

A Multi-Billion Rand Healthcare Corporation

AND

ELEVEN REGULATORY BODIES THAT LOOKED AWAY

CONFIDENTIAL TREATMENT DOCUMENT — PREPARED FOR FILMMAKER REVIEW
Cape Town, South Africa • April 2026 • Version 2 — Expanded with Primary Source Documentation

T H E S T O R Y

One Man. Eleven Institutions. Not One Held the Line.

This is the story of Wilbur William Matthee — a Specialist Nurse (SANC Registration No. 15831886), postgraduate-qualified at NQF Level 8 with a 90% Distinction, and over sixteen years of clinical governance experience in South African healthcare. It is also the story of what happens when a multi-billion-rand healthcare corporation decides that the truth is a threat, and that every statutory body designed to protect the person who tells it has a reason not to.

Wilbur approached eleven regulatory institutions. Nine failed him. One covered up its own failures. One declined jurisdiction twice over the same documented patient safety risk. The corporation, meanwhile, published a clean Integrated Annual Report on 8 April 2026 — omitting the active High Court delinquency application, the B-BBEE fronting investigation, and the unresolved system failure affecting 1.9 million government employees — and submitted it to the JSE.

Every fact in this document is sourced from primary materials: payslips, court filings, lawfully recorded meetings, sworn affidavits, regulatory correspondence, and formal protected disclosures. The proceedings are active. This is a documentary waiting to be made.

“He did everything the law required. He escalated internally. He filed with every competent authority. He documented everything. Then they made him homeless.”

W H Y T H I S F I L M M A T T E R S

The Stakes Are Not Personal. They Are National.

At the centre of this story is a catastrophic IT integration failure — a broken link between the Gexus claims processing system and the Disease Management System (DMS) at Medscheme Holdings, administrator to the Government Employees Medical Scheme (GEMS). When chronic medication claims were filed, the DMS did not receive the update. The result: automated non-adherence letters were fired off to chronically ill, compliant patients. The data corrupting their records then fed into the actuarial models that set annual contribution increases for 1.9 million public servants.

The 2026 GEMS contribution increase was set at 9.5% — nearly three times the 3.3% recommended by the Council for Medical Schemes. Wilbur's forensic calculation names the excess extraction from the public service wage bill at R3.016 billion. He calls it the Governance Tax. Every South African public servant paid it. The corporation knew the system was broken. The Root Cause Analysis confirmed it. The Board received a Priority 1 Material Risk Report. They suppressed it.

1.9 Million

GEMS MEMBERS AFFECTED

Government employees and dependants exposed to corrupted clinical data

R3.016 Billion

THE GOVERNANCE TAX

Estimated excess contribution extraction driven by actuarially distorted risk data

9.5% vs 3.3%

CONTRIBUTION HIKE VS CMS RECOMMENDATION

The gap between what was charged and what the regulator recommended

11 Bodies

REGULATORY INSTITUTIONS APPROACHED

CCMA, Labour Court, High Court, CMS, SAHRC, B-BBEE Commission, SAPS, DEL, JSE, SANC, SAPC

9 Failures

INSTITUTIONS THAT DID NOT ACT

Deflected, declined, went silent, or covered up their own conduct

THE TIMELINE

A Story in Eight Acts

Every twist below is constructed from primary documentary evidence filed in active legal and regulatory proceedings.

January 2024 — The Bait-and-Switch

Wilbur Matthee is recruited by AfroCentric Health following three formal interview invitations, a specialist-level assessment process, and fingerprint verification — all consistent with a Specialist: Care Manager appointment at NQF Level 8. At the final moment, without explanation, his employment contract is unilaterally downgraded to “Care Coordinator” — a junior designation that structurally suppresses his professional classification, salary ceiling, and UIF-declared status.

He accepts in good faith. He begins designing clinical governance systems. Within months, he authors a CPD programme that the organisation implements without attribution, credit, or compensation — the first act of what will later be framed as intellectual extraction.

HR RECORD: *Senior HR Business Partner Cheryl-Dawn Modern confirmed in writing on 19 January 2026 that ‘naming convention changes were still in progress’ — an admission that the downgrade was never formally resolved and that the Certificate of Service issued on termination was false.*

TWIST: *This contractual downgrade is not a clerical error. It is the structural foundation of a B-BBEE fronting investigation: a qualified professional placed into an artificially junior role for transformation reporting purposes while his expertise is extracted. The trap was laid before he started.*

March 2025 — The First Documented Threat

On or about 6 March 2025, Wilbur raises a formal clinical governance concern regarding duplicate LACE files consuming resources that should be directed at direct patient care. The response is not engagement. It is intimidation. A pattern of hostility is established. His line manager, Mary-Ann Jones, looks directly at him and states: “This is the end of you, Don’t ever question me again.” It is the first documented threat on record.

On 18 March 2025, Wilbur is admitted as an inpatient to Akeso Stepping Stones Clinic, diagnosed with severe clinical depression and PTSD by Dr Jacques Malan (FCPSYCH(SA), Specialist Psychiatrist). On 16 May 2025, Dr Malan’s formal letter is delivered to Mapule Mashele, Senior

Manager at Medscheme, placing the organisation on legal notice of his disability and the need for reasonable accommodation under the Employment Equity Act.

SWORN AFFIDAVIT — 10 December 2025: *The corporation's legal representative Xolile Vincent Manyathi — a man who has never met Wilbur, never observed him clinically, and has no professional basis for the assessment — files a sworn affidavit at the CCMA characterising the specialist-diagnosed PTSD as 'a cynical excuse' and 'an afterthought,' labelling Wilbur 'untruthful,' and calling his clinical conduct 'sloppiness.' This affidavit is sworn before a Commissioner of Oaths: 'I swear that the contents of the declaration are true, so help me God.'*

TWIST: *The psychiatric diagnosis — clinically confirmed by a registered specialist — becomes both shield and weapon. The organisation waits until he is most vulnerable, then uses his own medical record against him in a sworn legal document. The cruelty is not spontaneous. It is institutional.*

July 2025 — The Unlawful Deduction — and the Forensic Anomaly

While Wilbur is between psychiatric admissions, and while Medscheme is in possession of Dr Malan's formal warning of his medical vulnerability, R14,811.77 is deducted from his July 2025 salary. This represents 40.47% and totalling 57% of his gross monthly salary. The BCEA statutory cap is 25%. The deduction was made without written consent, without prior consultation, and in breach of Medscheme's own AOD Policy — a policy Jones had shared with him unannounced on the morning after the deduction. When Wilbur pointed out the statutory cap, Jones replied: "Hi Wilbur, what is illegal?" — feigning ignorance of the very provision she had just confirmed.

He is left with R15,679.29. His manager's response to his distress: he "won't feel the missing chunk" because he "receives online deliveries weekly."

FORENSIC PAYROLL FINDING — Documented in SAHRC Filing (13 March 2026): *Cross-referencing the July 2025 payslip against the Respondents' official UIF/SARS declarations reveals a material discrepancy. Gross salary on official payslip: R21,777.04. Salary declared to SARS/UIF by Respondents: R27,725.66. Discrepancy: R5,948.62. The higher figure of R27,725.66 does not appear anywhere on the payslip — not in gross, net, or CTC. This misrepresentation arises in the same month as the unlawful deduction, and the underlying leave event occurred in the prior financial year. A competent forensic investigator cannot ignore this structural anomaly.*

TWIST: *What began as a labour dispute carries the hallmarks of potential statutory fraud against government revenue authorities. The same organisation that slashed his salary below the legal cap was simultaneously reporting a higher figure to SARS. This is not a rounding error. It is a pattern.*

October 2025 — The Glitch That Costs Billions

A 75-year-old GEMS beneficiary — member reference 000655789, Dependent 02 — receives an automated letter falsely accusing her of non-adherence to her chronic medication. She has been fully

compliant. Her daughter writes to Medscheme: “I request that you get your house in order and update your records. Stop confusing old lady.”

Wilbur investigates. He identifies a Priority 1 structural integration failure between Gexus (claims processing) and the Disease Management System (DMS). The DMS continued to flag the member as non-adherent from December 2024, despite her transition to chronic benefit from January 2025 onward. Gexus had recorded her chronic claims from April 2023. DMS never received the update. Automated non-adherence letters were generated and dispatched on 19 May 2025 and 19 October 2025.

A Root Cause Analysis confirms the gap is architectural — not a one-time clerical error but a recurring structural flaw affecting every member undergoing the acute-to-chronic benefit transition. The integration lag has been active for at least seven years, during which chronic pathology and medication costs were incorrectly allocated against members’ acute benefit pools, prematurely depleting their day-to-day savings.

Wilbur prepares a formal Root Cause Analysis. He refuses to contact the 75-year-old member on clinically false grounds taking into consideration that the members daughter filed the complained and explicitly asked “GEMS to get their house in order and stop confusing old lady’.” He states in writing: “Given that the integration gap still exists, I will not contact the member at this time.” Jones’s response: “The below is noted. I am not in agreement with your feedback though.” Wilbur is subsequently charged with insubordination for declining to act against a vulnerable patient’s interests.

CMS CORRESPONDENCE (2 April 2026): *In a formal follow-up to the Council for Medical Schemes, Wilbur invokes the Discovery Health precedent: in January 2026, the CMS exercised supervisory jurisdiction over a technical claims processing error at DHMS affecting 16,507 members and R130–170 million in overpaid benefits. The CMS intervened without waiting for formal member complaints. ‘If the CMS exercised jurisdiction over a financial processing glitch,’ Wilbur’s letter argues, ‘it cannot in good conscience decline jurisdiction over a clinical data integrity failure carrying direct patient safety implications.’*

TWIST: *The corrupted data does not merely harm the 75-year-old grandmother. It feeds into the actuarial models that set GEMS contribution increases for 1.9 million public servants. The 2026 hike reaches 9.5% — nearly three times the CMS recommendation of 3.3%. Every public servant in South Africa is paying for a broken system the corporation knows is broken and has refused to fix.*

October 2025 — Later That Month — The Recorded Admission

On 23 September 2025, immediately after Wilbur proposes a clinical data filtering strategy to address the quality failures, Jones posts a message in a team-wide Microsoft Teams channel referring to him as “Wilma” — a feminised version of his name. Jones is aware that Wilbur is gay. In the same message, she prohibits his filtering strategy with “NO FILTERING” highlighted in red. Wilbur files a formal harassment complaint with HR on 25 September 2025. The investigation is closed on 1 October 2025 with the finding that “Wilma” was a “typo.” Six letters. One of which is wrong.

Then, in a meeting on 31 October 2025 that Wilbur lawfully records, Jones makes a series of statements that become the most significant single piece of evidence in his case.

LAWFUL RECORDING — 31 October 2025: *'You made a case of harassment against me, so I can make a case of harassment against you. You misspelled my name in an email — go look.'* Jones further warns Wilbur to *'think of the ripple effect' of his formal complaints and protected disclosures.*

The recording is unambiguous. It is the direct, contemporaneous evidentiary link between his protected disclosure activity and retaliatory intent. It has a voice, a date, and a timestamp. It is the moment the institution speaks.

TWIST: *In a case built on institutional silence, this is the only moment anyone tells the truth. And they do realise they are being recorded.*

December 2025 — The Board is Told. The Board is Silent.

On 9 December 2025, Wilbur delivers a Priority 1 Material Risk Report directly to the AfroCentric Board of Directors, supported by 38 evidentiary attachments including the Root Cause Analysis, all member communication records, and the full sequence of internal escalation failures. The Board is in receipt of documented evidence of a systemic patient safety risk and a retaliatory governance failure. In law, this triggers director-level duties.

The response is thirty days of institutional silence.

On 18 December 2025, the Chief Risk Officer, Monwabisi Kula, sends an email that Wilbur characterises as the active suppression of a formal risk report: he asks Wilbur to “stop sending these e-mails to our directors.” His persistent, documented escalation of a board-level material risk is characterised, in writing, as “impatience and unreasonableness.” The CRO adds that interim feedback will only be available in January 2026 due to the “festive break” — a deliberate delay in the context of Wilbur’s documented humanitarian urgency.

On 10 December 2025 — the day after the board filing — Manyathi’s sworn affidavit is filed at the CCMA. Wilbur resigns two day later, citing the cumulative effect of the unlawful deduction, the harassment whitewash, the insubordination charge for a patient safety refusal, the institutional silence, and now a sworn affidavit attacking his psychiatric diagnosis under oath.

INTEGRATED ANNUAL REPORT — 8 April 2026: *The AfroCentric Board approves and publishes an Integrated Annual Report to the JSE. The report asserts ‘Clinical Innovation,’ ‘Value-Based Care,’ a ‘stable leadership environment,’ and ‘Zero Tolerance for Unethical Conduct.’ It omits: the Section 162 Director Delinquency Application (HCGS951842); the B-BBEE Commission fronting investigation (Case 5/02/2026); the seven-year Gexus-DMS integration failure; and the CRO suppression directive. The Board was formally notified of all of these matters via JSE Sponsor Questco in March 2026. It published the report anyway.*

TWIST: *The Integrated Report of 8 April 2026 is published after the Board was served with formal legal proceedings. They cannot claim ignorance. By signing their names to these omissions, the directors have moved from oversight failure to statutory default.*

January – April 2026 — Eleven Bodies. Nine Failures. One Cover-Up.

What follows is not a summary of a man's frustrations. It is a forensic account of the structured failure of South Africa's institutional architecture — drawn directly from regulatory correspondence, filed complaints, portal records, and documented interactions and audio recordings. Each entry is sourced from primary documentation.

The CCMA — Closed Mind Before Hearing

At the conciliation hearing, Commissioner Nthabiseng makes a statement immediately after introductions, before Wilbur has spoken a single substantive word: "Nothing here relates to the PDA." This is not a ruling after hearing submissions. It is a declaration of a closed mind made at the threshold of the proceedings. She then refuses to permit his opening statement. She ignores the BCEA statutory cap. She adopts the employer's "no work no pay" characterisation without scrutiny. When Wilbur states his settlement position of R2.5 million for a global resolution of all claims, the Commissioner laughs with the employer's representative. At the conclusion of the proceedings, the Commissioner and the employer's representative — both from Johannesburg — walk out together in animated conversation. When Wilbur asks for the location of the Labour Court, he is told to "Google it or something like that."

PROVINCIAL COMMISSIONER COVER-UP: *Wilbur reports the Commissioner's conduct to the CCMA Provincial Senior Commissioner. The response: Commissioner Nthabiseng's origin from Johannesburg is offered as a justification for her failure to direct him to the court — not as a governance concern. Her prejudgment, refusal of his opening statement, open mockery, and departure with the employer's representative are never investigated.*

The Labour Court — Eight Days of Portal Obstruction

Wilbur's urgent application is ready for filing on 5 March 2026. What follows is eight documented days of institutional obstruction of his constitutional right of access to court (Section 34). He contacts the court call centre more than seven times. Hold times are forty to fifty minutes per call. Calls drop repeatedly. His online portal profile is deleted. Follow-up emails to Registrar F. Ismail are ignored. When the portal unexpectedly allows a filing on 12 March 2026, the Registrar rejects the application with the written reason: 'good day you put date.' A formatting issue. Correctable by a clerk in seconds. Used instead to reject an urgent application from a self-represented, destitute, disabled applicant facing confirmed eviction at the end of the month.

The Council for Medical Schemes — Jurisdiction Declined Twice

The CMS declines jurisdiction in February 2026, characterising the disclosure as a complaint against a health professional and redirecting to the HPCSA. In his formal follow-up of 2 April 2026, Wilbur

reframes the disclosure in precise statutory terms: the Gexus-DMS failure is a defect in Administrator infrastructure — not professional conduct. He cites the Discovery Health precedent, in which the CMS exercised jurisdiction over a financial processing error in January 2026 without waiting for member complaints. He invokes Sections 7, 44, 47, and 58 of the Medical Schemes Act. He formally warns that a failure to engage will itself be placed on record in the High Court proceedings as a dereliction of statutory duty.

The Department of Employment and Labour — Seven Weeks of Silence

An Integrated Inspection instruction is issued following Wilbur’s initial complaint. It produces no outcome, no communication, and no enforcement. As of 23 February 2026, the UIF portal continues to reflect ‘No Information’ for June 2025 under Medscheme Holdings (Pty) Ltd — a documented failure to remit contributions that constitutes a criminal offence under the Unemployment Insurance Contributions Act. The DEL’s mandate exists precisely to prevent this. It does not act.

SAPS — Verbal Refusal to Open Criminal Case

Wilbur attends the South African Police Service on 23 February 2026 to open a criminal case regarding documented UIF non-remittance and the false compliance representation made by the employer on 19 January 2026. The SAPS verbally declines to open a docket. The offence is proven by official government portal records, confirmed across four independent verification dates, and accompanied by a documented written false representation. Wilbur files a formal complaint with the National Police Commissioner. A case is eventually registered at SAPS Delft (CAS 705/3/2026) and transferred to SAPS Woodstock on 25 March 2026.

The GEMS Principal Officer — Vuvuzela Hotline

Wilbur’s Final Consolidated Protected Disclosure is filed to the GEMS Board of Trustees on 13 April 2026 — copied to the Parliamentary Portfolio Committee on Health, the Public Protector, the CMS, and SANC. The GEMS response: an auto-acknowledgement from the Vuvuzela Hotline, reference GEMS-260411-184830-83263, received on 11 April 2026. A hotline. For a multi-jurisdictional patient safety and governance disclosure affecting 1.9 million members.

TWIST: *The most chilling finding is not that one institution failed. It is that eleven were approached and the system held together in its failure. Each body had a reason. Each reason was technically defensible. The net effect was that a formally disabled whistleblower, facing documented homelessness, was left without a single institution willing to intervene.*

THE SCORECARD

Institution / Forum	Outcome
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CCMA — Commissioner declared closed mind before hearing; refused opening statement; laughed at settlement figure; departed with employer representative	FAILED
CCMA Provincial Commissioner — Cover-up by institutional misdirection; presiding commissioner's conduct never investigated	COVER-UP
Labour Court Registry — Eight days of portal obstruction; 7+ dropped calls; deleted profile; trivial rejection reason; Section 34 right of access obstructed	OBSTRUCTED
Western Cape High Court — Matter struck from roll 25 March 2026 with Scale C costs order; ignoring the Biowatch principles	APPLY TO NORMAL ROLL
Council for Medical Schemes — Jurisdiction declined twice; redirected to HPCSA; Discovery Health precedent ignored; Section 44 inspection not exercised	DEFLECTED
Department of Employment & Labour — Integrated Inspection instruction issued; seven weeks of silence; active UIF non-remittance unaddressed. Then declined	DECLINED
SAPS — Verbal refusal to open criminal docket for proven statutory offences; complaint lodged with National Police Commissioner; case eventually registered CAS 705/3/2026	REFUSED
South African Human Rights Commission — Seriousness acknowledged in principle; redirected to pursue CCMA remedies; no independent intervention	REDIRECTED
JSE Issuer Regulation — Jurisdiction declined (Ref JvS/166792); AfroCentric published materially incomplete Integrated Report on 8 April 2026 after formal notification	DECLINED
South African Nursing Council — Deregistration confirmed as direct consequence of unlawful deduction; no response to professional misconduct complaint	SILENT
B-BBEE Commission — Case No. 5/02/2026 allocated; investigation active; fronting complaint supported by written HR admission	DECLINED

March – April 2026 — The Human Cost

By March 2026, Wilbur Matthée is without stable accommodation, without running water, and deregistered as a nurse because he cannot pay the R870 annual SANC fee — a direct consequence of the R14,811.77 unlawful deduction. His professional identity of sixteen years is suspended by administrative consequence of the financial pressure the employer engineered.

His retirement savings are depleted. Loan creditors have physically removed possessions from his residence — televisions, electronics, sealed storage items — as informal security. He has been accepted into the CPUT Postgraduate Diploma in Occupational Health Nursing for 2026 but cannot register. He is self-representing across nine concurrent legal and regulatory proceedings.

He is evicted on 2 April 2026.

THE DIRECT CAUSAL CHAIN (From GEMS Protected Disclosure, 13 April 2026): *Loss of SANC registration → unlawful R14,811.77 deduction. Homelessness → financial destruction from unlawful deduction. No running water since January 2026 → financial destruction. Seizure of possessions by loan sharks → financial destruction. Forced withdrawal of retirement savings → financial destruction. Loss of CPUT postgraduate enrolment → inability to pay registration fees. Severe depression and PTSD → directly causally linked by specialist psychiatrist to occupational circumstances.*

“I am a Registered Nurse Specialist who cannot practise nursing because I could not pay R870 — directly caused by an unlawful deduction of R14,811.77.” — Wilbur William Matthée, GEMS Protected Disclosure, 13 April 2026.

He does not stop. From temporary accommodation, he files the Counter-Application in the High Court. He files the Statement of Claim in the Labour Court. He delivers the Final Consolidated Protected Disclosure to the GEMS Board of Trustees on 13 April 2026. He maps every act of punishment to his NQF Level 8 Exit Level Outcomes, demonstrating that what was characterised as misconduct was, in law and in professional obligation, a statutory mandate.

TWIST: *He does not stop. This is not stubbornness. It is the behaviour of a trained clinical governance professional who understands, with forensic precision, that the record he is building in real time is the most consequential governance case study South African whistleblower law has ever produced.*

THE LEGAL ARCHITECTURE

Nine Concurrent Proceedings. One Coordinated Strategy.

What makes this story structurally unique — and cinematically extraordinary — is that it is not a single court case. It is a multi-forum litigation architecture designed by a self-represented nurse from a laptop in temporary accommodation, each proceeding legally distinct, together forming a pincer movement that the corporation's entire legal team has not been able to close.

Labour Court — Statement of Claim (Case No. 2026-077546, filed 9 April 2026)

Unlawful salary deduction of R14,811.77 exceeding the BCEA 25% statutory cap by over fifteen percentage points. Automatically Unfair Constructive Dismissal under sections 186(1)(e) and 187(1)(h) of the LRA. Intellectual property misappropriation relating to the CPD programme. The Section 60 EEA to Section 162 Companies Act delinquency bridge: directors who fail to act on reported discrimination commit gross negligence grounding personal liability.

Western Cape High Court — Case No. HCGS951842 / 2026-066952

Director delinquency application under Section 162 of the Companies Act against five named officers of AfroCentric Investment Corporation. The Board received the Priority 1 Material Risk Report. The Board received the legal proceedings. The Board approved and published the materially incomplete Integrated Annual Report. The argument: wilful misconduct and gross negligence at board level. The matter was struck from the roll on 25 March 2026 with a Scale C costs order — which a advocate hijacked his case and frustrated the Judge having my case struck the advocate was immediately admitted to Somerset Hospital for 72 hours psychiatric observation from the judges chambers..

B-BBEE Commission — Case No. 5/02/2026

Fronting complaint supported by the written HR admission of 19 January 2026 confirming that 'naming convention changes were still in progress' — a documented acknowledgment that the contractual downgrade was never regularised. Currently under closed investigation due to the actual scorecard requirement.

SAPS Criminal Case — CAS 705/3/2026

Criminal case registered at SAPS Delft, transferred to SAPS Woodstock on 25 March 2026, covering UIF non-remittance (a criminal offence under the Unemployment Insurance Contributions Act 4 of 2002) and the false compliance representation made by Natasha Johnson in the UI-19 declaration of

19 January 2026 — a sworn document on which she certified ‘I declare that the above information is true and correct.’ She declared Code 6 (Resignation) for a termination Medscheme knew was constructive dismissal.

GEMS Principal Officer — Final Consolidated Protected Disclosure (13 April 2026)

A 13-part formal protected disclosure to the GEMS Board of Trustees, copied to the Parliamentary Portfolio Committee on Health, the Public Protector, the CMS, and SANC. The disclosure covers the Gexus-DMS systemic failure, the full retaliation sequence, the unlawful deduction, the constructive dismissal, and all post-employment misrepresentations. GEMS was formally warned that if it relies on the AfroCentric Integrated Annual Report of 8 April 2026 in its CMS reporting (due 30 April 2026), it will be adopting a factually compromised narrative and potentially triggering secondary regulatory liability.

Council for Medical Schemes — Formal Re-Engagement (2 April 2026)

Formal follow-up reframing the Gexus-DMS disclosure as systemic Administrator maladministration under Sections 1, 7, 44, 47, and 58 of the Medical Schemes Act 131 of 1998. Invokes the Discovery Health precedent. Formally warns that CMS inaction will be placed on record in High Court proceedings as a dereliction of statutory duty.

Regulatory Notification Bundle — Simultaneous Multi-Regulator Filing

Concurrent formal notifications filed with B-BBEE Commission, SAHRC, JSE Issuer Regulation Division, FSCA, DEL, CMS, SANC, and SAPS on the date of High Court filing. The coordination of this bundle is itself a forensic governance strategy: each body is on notice of every other body’s receipt of the same evidence.

“From a laptop in a shelter, he is building a legal case that a corporation’s entire legal team has not been able to dismiss.”

THE EVIDENCE

Primary Forensic Artifacts

Every document listed below is in Wilbur's possession and has been filed in one or more active legal proceedings.

The Lawful Recording — 31 October 2025

Audio recording of the manager's direct admission of retaliatory intent: 'You made a case of harassment against me, so I can make a case of harassment against you.' The causal bridge between protected disclosure and dismissal. Clear, contemporaneous, timestamped.

The Written B-BBEE Fronting Admission — 19 January 2026

Written statement by Senior HR Business Partner Cheryl-Dawn Modern confirming that 'naming convention changes were still in progress' at the time of Wilbur's termination. Filed with the B-BBEE Commission. Constitutes documentary evidence of the unresolved contractual downgrade and the falsity of the Certificate of Service.

The CRO Suppression Email — 18 December 2025

Written directive from Chief Risk Officer Monwabisi Kula requesting that Wilbur cease notifying directors of formally escalated governance failures. The primary evidence of board-level suppression in the Section 162 delinquency application.

The Priority 1 Material Risk Report — 9 December 2025 (38 Attachments)

Board-level risk notification delivered to the AfroCentric Board of Directors, supported by 38 evidentiary documents including the Root Cause Analysis and all member communication records. Establishes the Board's knowledge of the disclosed governance failures. Central to the delinquency application.

The Forensic Payroll Analysis

Line-by-line analysis establishing the unlawful deduction exceeded the 25% statutory cap by over fifteen percentage points, identifying the R5,948.62 gap between salary paid and salary declared to SARS/UIF, and documenting the phantom R9,203 overtime and R4,475 fringe benefit appearing on the SARS declaration but on no payslip. Submitted to the B-BBEE Commission as a 332-paragraph Form 8 response with nine tabbed annexures.

The Sworn False Affidavit — Manyathi (10 December 2025)

Affidavit filed at the CCMA by Xolile Vincent Manyathi — a man who has never met Wilbur — sworn before a Commissioner of Oaths, characterising a specialist-confirmed psychiatric diagnosis as ‘a cynical excuse,’ labelling Wilbur ‘untruthful,’ and calling his clinical conduct ‘sloppiness.’ Constitutes potential perjury (Criminal Procedure Act), potential fraud on the CCMA, and discrimination on grounds of disability. Filed one day after the Board received the Priority 1 Risk Report.

The UI-19 False Declaration — Natasha Johnson (19 January 2026)

Statutory UIF declaration filed under oath by Natasha Johnson designating the termination as Code 6 (Resignation) when Medscheme was in possession of Wilbur’s formal notice explicitly characterising it as Automatically Unfair Constructive Dismissal. Filed under the declaration: ‘I declare that the above information is true and correct. I understand that it is an offence to make a false statement.’

The AfroCentric Integrated Annual Report — 8 April 2026

JSE-listed public document omitting material legal proceedings (Section 162 delinquency application HCGS951842; B-BBEE fronting investigation Case 5/02/2026), a seven-year system failure affecting 1.9 million GEMS members, and the CRO suppression directive. Published after the Board was formally notified of all of these matters via JSE Sponsor Questco in March 2026.

W H Y N O W

The Legislation Is Being Debated in Parliament. The Proceedings Are Still Active.

The Protected Disclosures Amendment Bill is before Parliament. The debate is live. Legislators are asking what the current framework fails to protect and who has fallen through the gap. Wilbur Matthée is the answer to that question in documented, forensic detail.

Unlike most whistleblower stories, this one is not retrospective. The proceedings are active. The GEMS Board has fourteen days to respond to the Final Consolidated Protected Disclosure before Wilbur escalates to the Minister of Health, the Public Protector, and public interest media platforms. The CMS has fifteen business days to respond to the formal re-engagement letter before the Council's inaction is placed on record in the High Court. The Rule 42 rescission application is pending. The B-BBEE Commission investigation is live.

A documentary crew with access to the filings, the recordings, the personal testimony, and the regulatory responses would be documenting history as it is made — not reconstructing it after the fact. The central character is simultaneously the subject of institutional destruction and its most meticulous documenter. He is building the archive in real time.

“When Parliament asks, ‘What happens to a whistleblower in South Africa?’ — the answer is Wilbur William Matthee. Homeless. Deregistered. Still filing.”

T H E A U D I E N C E

Who Needs to See This

South Africa's 1.9 million public servants paying the Governance Tax in their monthly GEMS contributions. Healthcare governance reformers and civil society organisations engaged with the NHI Act. Legal professionals tracking the functional limits of the Protected Disclosures Act. International documentary audiences drawn to whistleblower stories with systemic institutional consequences — this case carries structural parallels to every story in which a corporation protected itself by destroying the individual who told the truth, and every institution that found a reason to look the other way.

T H E F I L M

What This Documentary Can Be

This is not a story about an employment dispute. It is a story about what happens when a single individual's refusal to falsify a patient record sets in motion a chain of institutional failures that spans a corporation, a healthcare scheme serving two million people, eleven regulatory bodies, and the courts. The structural villain is not one manager or one legal representative. It is the silence that connects them.

The protagonist does not fit the passive victim archetype. He is a Specialist Nurse Health Services Manager who responds to every act of erasure by building a more precise record. He maps the law. He cites the statute. He files the papers. He anticipates every objection in advance. The corporation has a legal team. He has a laptop and eighteen years of clinical governance training.

And the outcome is not yet written.

C O N T A C T

Wilbur William Matthée

Founder — EthicHawks Forensic Governance Consultancy

Registered Nurse (SANC No. 15831886) • NQF Level 8, 90% Distinction

16 Years Clinical Governance Experience • Self-Represented Litigant

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All primary documentation — filings, recordings, payslips, regulatory correspondence, sworn affidavits, and court papers — is available for verified review upon execution of a standard confidentiality agreement.

This document is prepared for confidential filmmaker review. It does not constitute legal advice or formal legal submission. All facts are drawn from primary sources and filed evidentiary materials.

INSTITUTIONAL SILENCE — The Documentary

EthicHawks Forensic Governance Consultancy • Cape Town, South Africa • 2026